



Gender: Female

DOB:

Race: White

Report Date:

Pathology Report:

ICD - 0 - 3
Carcinoma, urothelial, NOS 8120/3
CQCF Site: bladder, wall, posterior C67.4
Path. Site: bladder, wall, NOS C67.9

Surgical Pathology Report

FINAL PATHOLOGIC DIAGNOSIS

A. Right pelvic lymph node; dissection:
- Fifteen lymph nodes, no tumor (0/15).

B. Left pelvic lymph node; dissection:
- Eleven lymph nodes, no tumor (0/11).

C. Urinary bladder; cystectomy:
- Invasive high grade urothelial carcinoma with squamous differentiation, see pathologic parameters.
- Tumor invades thru the bladder wall and into the perivesical fat.
- All margins of resection, free of tumor.

D. Distal left ureter; excision:
- Portion of ureter, no tumor.

E. Distal right ureter; excision:
- Portion of ureter, no tumor.

Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
IIPAA Discrepancy		X
Prior Malignancy History	X	
Dual/Synchronous Primary Noted		X
Case (single):	QUALIFIED	DISQUALIFIED
Reviewed by:	8/17/12	

Urothelial Carcinoma Pathologic Parameters

1. Tumor type: Invasive urothelial carcinoma with squamous differentiation.
2. Grade of tumor: High grade. < 10% Squamous per TSS. 4/7/12
3. Depth of invasion: Extravesicular soft tissue (macroscopic).
4. Tumor distribution: Solitary mass, 2.5cm, involving posterior wall, left lateral wall, and trigone.
5. Ureteral margins: Negative for tumor.
6. Distal urethral margin: Negative for tumor.
7. Soft tissue margin or serosa: Negative for tumor.
8. Lymph nodes: Negative for tumor (0/26).
9. pTNM: pT3b,N0,MX.

this Checklist utilizes the 7th edition TNM staging

system for Bladder of the American Joint Committee on Cancer (AJCC) and the International Union Against Cancer (UICC).

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[] MD

Interpretation performed by the Attending Pathologist and reviewed with the Resident/Fellow, [], M.D.

Electronically Signed Out by [] MD

Clinical History:

This is a -year-old woman with a history of papillary urothelial carcinoma, high grade invading into the muscularis propria, now undergoing cystectomy.

Specimens Received:

A: Right pelvic lymph node

B: Left pelvic lymph node

C: Bladder

D: Distal left ureter

E: Distal right ureter

Gross Description:

The specimens are received in five containers each labeled with the patient's name and medical record number.

A. The first container is additionally identified as, "1. Right pelvic lymph node". Received fresh and placed in formalin is a lobulated yellow and brown, 5.7 x 4.8 x 2 cm tissue from which multiple lymph node candidates measuring 0.2-2.5 cm are identified.

A1: 4 candidate lymph nodes

A2: 5 candidate lymph nodes

A3: 4 candidate lymph nodes

A4: 1 candidate lymph node, bisected

A5-A6: One candidate lymph node, bisected

B. The second container is additionally identified as, "2. Left pelvic lymph node". Received fresh and placed in formalin is a lobulated yellow and dark brown 7 x 4.5 x 1.5 cm tissue from which multiple lymph node candidates are identified measuring 0.3-3.3 cm.

B1: 5 candidate lymph nodes

B2: 4 candidate lymph nodes

B3: One candidate lymph node, bisected

B4-B5: One candidate lymph node, bisected

B6: Section of fibroadipose tissue

C. The third container is additionally identified as, "3. Bladder". Received fresh and placed in formalin is a 150 g cystectomy specimen with overall measurements of 13.5 x 12 x 3.8 cm. The right ureteral stump measures 2.5 x 0.5 cm, and the left measures 1.8 x 0.4 cm. The right ureter is patent to the surgical margin. The serosal surface shows yellow lobulated fibroadipose tissue with focal cautery artifact. The urethral margin is inked black, and the bladder is inked blue on the right half (including right ureteral stump) and black on the left half. The specimen is opened anteriorly to reveal a 2.5 x 2.0 cm centrally ulcerated mass occupying the posterior wall and left lateral wall extending to within 0.5 cm from the left ureter orifice. The left ureter orifice is only patent in the most proximal 0.5 cm. On cut section, the mass is firm and displays tan-yellow cut surface with focal hemorrhagic discoloration. The mass extends into and through the muscularis propria and is close to the black ink. The surrounding bladder mucosa is wrinkled and pink-tan with a wall thickness of 1.6-2.1 cm.

Representative sections are submitted as follows:

C1: Distal urethral margin

C2: Right ureter resection margin (blue) and left ureter resection margin

C3-C6: Closest extension of mass to ink

C7: Mass in trigone

C8: Section of mass and adjacent left ureteral orifice

C9: Uninvolved anterior wall

C10: Uninvolved dome

C11: Uninvolved right lateral wall

D. The fourth container is additionally identified as, "4. Distal left ureter, suture on non-margin side". Received fresh and placed in formalin is an oriented square shaped 0.8 x 0.4 x 0.2 cm portion of tissue with dark brown and yellow tissue on one surface and pale tan-pink apparent mucosa on the other surface. A lumen is not identified. The margin side is trimmed and submitted as D1. The remaining tissue is free of tumor involvement grossly..

E. The fifth container is additionally identified as, "5. Distal right ureter, suture on non-margin side". Received fresh and placed in formalin is an oriented, dark brown and yellow segment of ureter measuring 2.1 x 0.7 x 0.5 cm. The margin side is trimmed and submitted as E1. The remainder of the specimen is sectioned revealing maximal wall thickness of 0.2 cm and a pinpoint lumen with no gross involvement by tumor.

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[] M.D.